

DELETE THIS SLIDE BEFORE PRESENTING

Setup checklist for the person preparing this deck

1. **Fill in the placeholders.** Every {angle bracket} token in the slides, the speaker notes and the sign-off sheet is filled in from your own policy before the talk.
2. **Keep the slides flagged for your jurisdiction.** Two slides come in three versions — Northern Ireland, Republic of Ireland and Great Britain — labelled top right. Keep the one that matches where you operate and delete the other two.
3. **Delete this slide.** Once the checklist is done, remove it so the deck opens on the cover.
4. **Print the sign-off sheet.** One sheet, with enough rows for everyone attending, plus a pen that works.
5. **Test the video with the sound on.** The short procedure film sits in the testing section. Play it once before the session, on the screen and speakers you will use.
6. **Allow 25 to 35 minutes.** That is the run time for the talk itself, including the film and the three questions at the end.



Toolbox talk

Drugs and alcohol at work

⟨Company name⟩ · ⟨Date⟩

Delivered by ⟨Presenter⟩ · about 25 to 35 minutes

Prepared with NIVHA Laboratory Services

Sign-off sheet accompanies this deck. The full delivery script is in the speaker notes.

Why we are talking about this

Safety, not surveillance. The policy asks one question about today.

The policy asks one question: are you fit to do your work safely today?

Not about

Your weekend, your private life, or judging anybody's choices. No moral test is being applied here.

About

Impairment at work, on this site, today — and the people working beside you who depend on you being sharp.

And about support

A route to ask for help before something goes wrong, which is not the same route as discipline.

- Your employer must protect the health, safety and welfare of everyone at work, so far as is reasonably practicable
- You must not be under the influence of an intoxicant to the extent that you endanger yourself or anyone else

Source: [HSE — the law](#) · [Safety, Health and Welfare at Work Act 2005, s.13](#) · [HSA — intoxicants at work](#)

The scale of it

Official Northern Ireland figures, so that nobody thinks this is a rare problem somewhere else.

251

drug-related deaths registered in Northern Ireland in 2024, of which 219 met the definition of drug misuse

101

of those 251 certificates named pregabalin, a prescription medicine — the single drug named most often

131

certificates named opioids, the drug group appearing most often, and the 35 to 44 age group had the highest rate

These are working-age people. On the numbers, in any large workforce there will be people affected, and most will believe it never touches their work.

Source: [NISRA](#)

The scale of it

Official Republic of Ireland figures, so that nobody thinks this is a rare problem somewhere else.

343

drug poisoning deaths recorded in Ireland in 2022, the second consecutive annual fall

33.5%

of those deaths had cocaine implicated — around one in three

259%

rise in cocaine poisoning deaths between 2013 and 2022, the largest rise of any drug group

These are working-age people. On the numbers, in any large workforce there will be people affected, and most will believe it never touches their work.

Source: [HRB National Drug-Related Deaths Index](#) · [HRB deaths data](#) · [EUDA European Drug Report 2026 — cocaine](#)

The scale of it

Official figures for England and Wales, so that nobody thinks this is a rare problem somewhere else.

5,565

deaths related to drug poisoning
registered in England and Wales in 2024 —
the highest number since the series began
in 1993

1,279

of those deaths involved cocaine, a rise of
14.4% in a single year

2012

the year from which the age-standardised
rate has risen every year since

These are working-age people. On the numbers, in any large workforce there will be people affected, and most will believe it never touches their work.

The published series covers England and Wales; Scotland reports separately.

Source: [ONS](#)

What the policy counts as a substance

Four groups, and the last two surprise people.

Alcohol

The most used substance in the population. It is in scope during working hours and paid breaks, and it can still be affecting somebody the morning after.

Illegal drugs

Cocaine is the substance found most often in workplace samples, with cannabis second. Possession or use on site is a policy matter in every case.

Prescription and pharmacy medicines

Only when they are misused, or when a medicine that affects alertness has not been declared. Codeine-containing painkillers, sedating antihistamines, sleeping tablets and prescribed sedatives all count.

Psychoactive substances

So-called legal highs and novel substances. The contents are unknown to the person taking them, which makes the effect at work unpredictable.

A declared, prescribed medicine handled through medical review is not a policy breach. An undeclared one is a different conversation.

What impairment actually looks like on site

Plain terms. None of this needs a clinical explanation.

Judgement

Risks look smaller than they are. Shortcuts start to feel reasonable, and the decision to stop feels unnecessary.

Reaction time

The gap between seeing something and doing something about it stretches. Around moving plant, that gap is the whole margin of safety.

Coordination and balance

Grip, footing and fine control slip. Working at height, on ladders or with tools, that shows up quickly.

Fatigue and attention

Concentration drifts, and tiredness after use can last well beyond the effect itself. Repetitive tasks are where it bites.

The person affected is usually the last one to notice — the standard is fitness for work, not self-assessment.

Source: [HSE — managing drug and alcohol misuse at work](#)

Our policy, and the rules here

The standard is fitness for work, and it applies to everybody on site.

- The standard is fitness for work: if alcohol, drugs or any substance could be affecting you, you do not start
- Working hours and paid breaks are both covered, and it applies to everyone on site including contractors and visitors
- Testing that applies here: {Testing types in your policy}
- Breaches are treated seriously, and the policy sets out what follows a non-negative screen
- The policy owner is {Policy owner} and the full document is available at {Where the policy is published}

Read the policy once today, rather than for the first time on a bad day.

Testing types, the support stance and the policy location are filled from the policy answers.

How a test actually works

A short film of an oral fluid collection, start to finish, then the paper trail behind it.

Oral fluid collection — procedure film

Open the film: youtu.be/_2tAEqwoOml

Watch for four moments: the sealed kit opened in front of the donor, how the swab is handled, the tamper-evident seal going on, and the paperwork being signed.

Chain of custody — the paper trail that protects you

- A sealed kit is opened in front of you and the sample is sealed while you watch
- You check the details, sign the chain of custody form and keep a copy
- Every handover is logged; if any link in the chain is broken, the result is not defensible

Nobody appearing in the film is an employee of any client organisation.

A negative screen ends the matter and you go back to work.

Screening and confirmation are different things

A screen indicates. Only the laboratory confirms.

Screening, on site

A point-of-care device gives an indication. It is not evidential, it is not presented as final, and it is never described as a failed test. The word used is “non-negative”.

Confirmation, in the laboratory

Further samples, different analytical methods, assessed against the current published cut-off schedule of the analysing laboratory. This is the evidential step, and it produces a certificate.

A non-negative screen is not a failed test. It is the point at which the sample goes to a laboratory.

- You are stood down from safety-critical work while the process runs — a precaution, not a finding
- A further sample is retained so that independent re-analysis can be requested
- Confirmation is carried out by an accredited laboratory, and only that step produces a certificate

Medication: declare it, and read the label

Declared in advance, a prescribed medicine is not a policy breach.

Declare before, not after

Use the confidential route — {Medical declaration route} — before a test and before starting safety-critical work on something new. You never have to tell your line manager what the condition is.

Read the label

“May cause drowsiness”, “do not drive”, “do not operate machinery”. Those warnings describe the work many of you do. Raise it so the task can be looked at.

Prescribed cannabis-based products

They are medication and go through medical review, where a clinician assesses the laboratory finding against the declared prescription.

CBD products

These are not tightly regulated and some contain more than the label claims. If you use one, declare it through the same confidential route.

Clinical detail goes to the reviewing clinician, not to management. The authorised contact receives an outcome only.

The confidential declaration route is filled from the policy answers.

Your rights through the process

Consent, dignity, confidentiality, and who is allowed to see a result.

Consent

A sample is given with your agreement, after the procedure has been explained. You may ask for it to be explained again at any point.

Dignity

A private room is used, never the canteen or the shop floor.
Nobody supervises you in a way that would embarrass you.

Confidentiality

A result is health data, a special category with stronger protection. Only the authorised contacts named in the policy see it.

Your information

You can ask what is held about you, ask for a copy, and ask for inaccurate information to be corrected. Records are kept only for the period the policy sets out.

A supervisor is told whether you are fit for work. A supervisor is not told what was found.

Refusing a test, and interfering with a sample

Both are treated as seriously as a confirmed positive result.

- 01 Refusing to provide a sample without good reason is generally treated the same as a positive result
- 02 Walking away, delaying until the collector leaves, or leaving site when asked are all treated as refusal
- 03 Substituting, diluting or adding anything to a sample is treated as tampering, and the attempt is the issue
- 04 Collection kits and laboratory checks are designed to reveal interference with a sample
- 05 If you have a genuine reason you cannot provide a sample, say so at the time so that it can be recorded

Nobody is asked to prove they are innocent. The process is there so that decisions rest on evidence rather than an opinion on site.

Where to go if this is becoming a problem

Take a photograph of this slide. Asking for help is not a breach of the policy.

Come forward first

Self-referral before you are selected for testing is treated as a health matter, with support. Coming forward is not the same as being found out, and the policy recognises the difference.

Confidentially, here

Speak to (Support contact), or use the employee assistance programme where one is in place. This is a health route, not a disciplinary one.

Your GP

Your GP can treat this as the health matter it is, and that route involves your employer not at all.

Public services

The services listed on nidirect, and the addiction services run by Inspire. Both are open to anybody, at any stage, and neither goes through your employer.

Asking for help is not a breach of any policy worth having.

The employee assistance programme line is shown only when a programme is in place.

Source: [nidirect — getting help with drug or alcohol problems](#) · [Inspire — addiction services](#)

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Your GP can treat this as the health matter it is, and that route involves your employer not at all.

Public services

The HSE Drugs and Alcohol Helpline. It is open to anybody, at any stage, and it does not go through your employer.

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The employee assistance programme line is shown only when a programme is in place.

Source: [HSE Drugs and Alcohol Helpline](#)

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Your GP can treat this as the health matter it is, and that route involves your employer not at all.

Public services

The national drug advice service FRANK, and Drinkaware for alcohol. Both are open to anybody, at any stage, and neither goes through your employer.

Asking for help is not a breach of any policy worth having.

Add the current contact details for FRANK and Drinkaware before you present this slide.

Source: [HSE — managing drug and alcohol misuse at work](#)

Three quick questions

Hands up, with the answer read out straight after each one. Nothing is scored or recorded.

01 What single question is the policy asking of you?

02 What does a non-negative screening result mean?

03 When do you declare medication, and what do you not have to say?

Answers are in the speaker notes for this slide.

Close

Everyone who comes to work healthy and well goes home the same way.

- The policy asks one question: are you fit to do your work safely today?
- A screening device indicates, the laboratory confirms, a clinician reviews declared medication
- Declare your medication, and ask for help early — neither one is a breach

Please sign the sign-off sheet before you leave. Any question left unanswered is written on the sheet and passed to {Policy owner}.

